

SUNNAH REMEDIES ACADEMY

Institute of Prophetic Medicine

PHASE 1

THE COMPLETE ACADEMY BLUEPRINT

Version 2.0 — Hijama Practitioner Certification Programme

SECTION A: INSTITUTIONAL FOUNDATION

A.1 Mission

To restore Hijama (wet cupping therapy) to its rightful place as a distinguished clinical discipline — grounded in authentic Prophetic guidance, informed by contemporary evidence, and practised with the highest standards of safety, ethics, and professional excellence.

We exist to produce practitioners who are simultaneously:

- Clinically competent and legally compliant
 - Spiritually grounded and ethically exemplary
 - Evidence-informed and traditionally respectful
 - Patient-centred and professionally outstanding
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A.2 Vision

To become the world's premier institute for Hijama education — the institution against which all other programmes are measured.

Within five years, Sunnah Remedies Academy will:

- Graduate practitioners across six continents who are recognised for clinical excellence
 - Publish original research contributing to the global evidence base for cupping therapy
 - Establish partnerships with universities, hospitals, and Islamic institutions worldwide
 - Create a living curriculum that evolves with science while honouring tradition
 - Build a global community of practitioners committed to continuous improvement
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A.3 Graduate Profile

Upon certification, a Sunnah Remedies Hijama Practitioner will demonstrate:

Clinical Competence

- Safe, effective, and legally compliant wet cupping practice
- Systematic patient assessment and clinical reasoning
- Infection prevention and control at clinical standards
- Adverse event recognition and emergency management
- Accurate documentation and professional record-keeping

Islamic Grounding

- Sound understanding of Hijama in Qur'anic and Prophetic tradition
- Ability to distinguish between revelation, authentic Sunnah, scholarly interpretation, and clinical practice
- Ethical practice informed by Islamic principles of amanah (trustworthiness), ihsan (excellence), and rahmah (compassion)
- Appropriate communication of Islamic dimensions to patients of all backgrounds

Evidence Literacy

- Critical appraisal of research literature on cupping therapy
- Honest communication of evidence strengths and limitations to patients
- Integration of traditional practice with contemporary scientific understanding
- Awareness of emerging research and evolving best practice

Professional Excellence

- Commitment to continuing professional development
- Ethical marketing and patient communication
- Interprofessional collaboration and appropriate referral
- Business competence for independent practice or employment

Personal Qualities

- Humility in the presence of knowledge — both sacred and scientific
- Compassion as the foundation of patient care
- Integrity in every clinical and business decision
- Continuous self-reflection and improvement

A.4 Core Values

Ihsan — Excellence in Everything We do not accept “good enough.” Every slide, every explanation, every clinical protocol is crafted to the highest standard. We believe the Prophet’s instruction to “do things with excellence” (hadith) applies to education as much as to practice.

Amanah — Trustworthiness Our students trust us with their education, their time, and their aspirations. We honour that trust with accuracy, honesty, and rigour. We never teach what we cannot verify.

Rahmah — Compassion Every patient who enters a Sunnah Remedies graduate’s clinic will be treated with dignity, gentleness, and genuine care. Compassion is not an add-on — it is the foundation.

Ilm — Knowledge with Humility We pursue knowledge relentlessly — both the revealed knowledge of Islam and the discovered knowledge of science. We hold both with humility, recognising that all true knowledge comes from Allah.

Itqan — Mastery through Craft We believe that mastery comes through deliberate practice, continuous refinement, and attention to detail. Our graduates are craftsmen of clinical care.

SECTION B: CURRICULUM ARCHITECTURE

B.1 Programme Overview

Attribute	Specification
Programme Title	Hijama Practitioner Certification Programme
Award	Certified Hijama Practitioner (CHP)
Accreditation	CMA & IPHM Accredited
Total Hours	120 guided learning hours + 40 supervised practice hours
Duration	12 weeks (part-time) or 4 weeks (intensive)
Delivery	Blended: live online lectures + in-person practical workshops + self-directed study
Assessment	Written examination (40%) + Practical OSCE (40%) + Portfolio (20%)
Prerequisites	Minimum age 18; basic literacy in English; no prior clinical qualification required
Maximum Class Size	16 students (to ensure individual practical attention)

B.2 Learning Outcomes

Upon successful completion, graduates will be able to:

Domain 1: Knowledge (K)

K1. Explain the historical development of cupping therapy across civilisations with scholarly accuracy
K2. Articulate the Islamic foundations of Hijama with proper referencing of Qur'an, authentic Hadith, and classical scholarship
K3. Describe the anatomy and physiology relevant to cupping practice with clinical precision
K4. Summarise the current evidence base for cupping therapy, including strengths, limitations, and gaps
K5. Explain the mechanisms of action proposed for cupping therapy, distinguishing between established and theoretical pathways
K6. Identify contraindications, precautions, and risk factors for cupping therapy
K7. Describe infection prevention and control standards applicable to wet cupping practice

Domain 2: Clinical Skills (S)

S1. Conduct a comprehensive pre-treatment patient assessment including history, examination, and risk stratification
S2. Obtain valid informed consent through clear communication of benefits, risks, and alternatives
S3. Perform wet cupping safely using correct technique, equipment, and infection control measures
S4. Apply appropriate cup placement based on clinical reasoning, anatomical knowledge, and patient presentation
S5. Manage common adverse events and recognise when emergency intervention is required
S6. Provide appropriate aftercare advice and follow-up planning
S7. Maintain accurate clinical documentation in compliance with legal and professional requirements

Domain 3: Professional Practice (P)

P1. Practice within scope of competence and recognise when referral to other healthcare professionals is appropriate
P2. Apply ethical principles derived from both Islamic teaching and professional healthcare standards
P3. Communicate effectively with patients from diverse backgrounds, including sensitive discussion of Islamic dimensions
P4. Maintain professional indemnity insurance and comply with all legal requirements for practice
P5. Commit to continuing professional development and evidence-based practice
P6. Operate a Hijama clinic or practice within an existing healthcare setting with business competence

Domain 4: Islamic Integration (I)

I1. Distinguish accurately between Qur'anic teaching, authentic Sunnah, scholarly interpretation, traditional practice, and scientific evidence
I2. Integrate Islamic ethical principles naturally into clinical practice
I3. Communicate the Islamic dimensions of Hijama with accuracy, respect, and appropriate context
I4. Demonstrate personal commitment to the spiritual dimensions of healing practice

B.3 Competency Framework

The programme maps to six competency domains, each with progressive levels:

Domain	Level 1: Foundational	Level 2: Developing	Level 3: Competent	Level 4: Proficient
Knowledge	Recalls facts	Explains concepts	Applies knowledge	Synthesises and teaches
Clinical Assessment	Identifies components	Performs with guidance	Performs independently	Adapts to complex cases

Table 2 – continued

Domain	Level 1: Foundational	Level 2: Developing	Level 3: Competent	Level 4: Proficient
Technical Skills	Observes and describes	Practises with supervision	Performs competently	Refines and innovates
Safety Management	Recognises hazards	Applies protocols	Manages incidents	Prevents and educates
Communication	Listens actively	Communicates clearly	Adapts to context	Inspires and mentors
Professionalism	Understands ethics	Applies principles	Models excellence	Leads and influences

B.4 Curriculum Map

Module Structure

Module	Title	Hours	Learning Domains
M1	Foundations of Hijama	16	K1, K2, I1, I2
M2	The Science of Hijama	16	K4, K5, K6
M3	Human Anatomy for Practitioners	20	K3, S4
M4	Clinical Practice & Safety	20	K7, S1, S2, P1, P2, P4
M5	Treatment Protocols	24	S3, S4, S5, S6, S7
M6	Professional Standards & Practice	16	P3, P5, P6, I3, I4
Practical Workshops	Supervised Clinical Practice	40	S1-S7, P1-P6
Assessment	Written + OSCE + Portfolio	8	All domains

Module Sequencing Rationale

The curriculum follows a deliberate pedagogical progression:

1. **Why Hijama matters** (Module 1) — establishes purpose, motivation, and Islamic foundation
2. **How it works** (Module 2) — builds scientific understanding and evidence literacy
3. **What the body looks like** (Module 3) — develops anatomical knowledge essential for safe practice
4. **How to keep patients safe** (Module 4) — establishes safety as the non-negotiable foundation
5. **How to perform Hijama** (Module 5) — teaches the actual clinical procedure with precision
6. **How to build a practice** (Module 6) — prepares graduates for professional life

This sequence ensures students understand WHY before HOW, and SAFETY before TECHNIQUE.

SECTION C: MODULE DETAILS

MODULE 1: Foundations of Hijama

16 guided learning hours | 4 lessons

Lesson 1.1 — The History of Cupping Therapy (4 hours)

Learning Objectives:

- Trace the documented history of cupping from ancient Egypt to the present day
- Identify contributions from Egyptian, Chinese, Greek, and Islamic civilisations
- Evaluate historical sources with scholarly discrimination

Content:

- Ancient Egypt: Ebers Papyrus (c. 1550 BCE), archaeological evidence
- Ancient China: Documentation from 281 CE, evolution of instruments
- Ancient Greece: Hippocratic corpus, clinical descriptions
- Islamic Golden Age: Ibn Sina, Al-Zahrawi, Al-Razi — their contributions to cupping literature
- The Prophet Muhammad (peace be upon him) and Hijama: authentic narrations and practice
- Evolution of instruments: animal horns to modern disposable systems
- Contemporary revival: global spread and institutional development

Signature Elements:

- Sunnah Remedies Historical Timeline
- Sunnah Remedies Islamic Insight: The Angels and Hijama
- Sunnah Remedies Evidence Update: Archaeological findings 2020-2025
- Faith & Practice Reflection

Lesson 1.2 — Hijama in the Islamic Tradition (4 hours)

Learning Objectives:

- Quote and reference Qur'anic verses related to healing and medicine
- Cite authentic Hadith concerning Hijama with proper sourcing
- Explain the scholarly tradition of Tibb al-Nabawi (Prophetic Medicine)
- Distinguish between strong and weak narrations regarding Hijama

Content:

- Qur'anic foundation: Surah Al-Shu'ara (26:80) and related verses
- Authentic Hadith: Bukhari 5357, 5371, 5680, 5694, 5697; Muslim 5706, 2952
- The Prophet's own practice: locations, timing, and circumstances
- Scholarly tradition: Ibn al-Qayyim's Zad al-Ma'ad, Al-Tibb al-Nabawi
- Classical scholarly positions on Hijama: Imam Malik, Imam Ahmad, Imam al-Nawawi
- The Isra and Mi'raj narrations: scholarly analysis of authenticity

- Sunnah points: Al-Kahil, Akhda'een, Al-Hamma, Umm al-Muqeeth, Al-Katifain

Signature Elements:

- Sunnah Remedies Islamic Insight: Hadith authentication framework
- Sunnah Remedies Scholarly Commentary: Ibn al-Qayyim on Hijama
- Sunnah Remedies Evidence Update: Contemporary scholarly positions
- Faith & Practice Reflection

Lesson 1.3 — Defining Hijama: Wet vs. Dry Cupping (4 hours)

Learning Objectives:

- Define wet cupping (Hijama) with clinical precision
- Distinguish between wet cupping, dry cupping, and other modalities
- Explain the regulatory status of cupping therapy in key jurisdictions

Content:

- Definition of wet cupping (Hijama): controlled incision + negative pressure + blood extraction
- Dry cupping: suction without incision
- Fire cupping, massage cupping, flash cupping, herbal cupping, needle cupping
- Regulatory landscape: UK, USA, Canada, Australia, UAE, KSA
- Scope of practice: what Hijama practitioners can and cannot do
- Professional associations and accreditation bodies

Signature Elements:

- Sunnah Remedies Clinical Pearl: The Hijama Definition Framework
- Sunnah Remedies Comparison Table: All cupping modalities
- Sunnah Remedies Practitioner Tip: Explaining the difference to patients
- Faith & Practice Reflection

Lesson 1.4 — The Hijama Practitioner: Role and Responsibility (4 hours)

Learning Objectives:

- Articulate the professional identity of a Hijama practitioner
- Understand the ethical framework governing practice
- Develop a personal professional development plan

Content:

- The practitioner as amanah-holder (trustee)
- Ihsan in clinical practice: excellence as worship
- Scope of practice and professional boundaries
- Interprofessional relationships and appropriate referral

- Personal health and wellbeing for practitioners
- Building a continuous learning mindset

Signature Elements:

- Sunnah Remedies Islamic Insight: The Ethics of Healing
 - Sunnah Remedies Professional Standards: The Amanah Framework
 - Sunnah Remedies Reflect & Apply: Personal development planning
 - Faith & Practice Reflection
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MODULE 2: The Science of Hijama

16 guided learning hours | 4 lessons

Lesson 2.1 — Research Evidence: Systematic Reviews and Meta-Analyses (4 hours)

Learning Objectives:

- Navigate the peer-reviewed literature on cupping therapy
- Evaluate evidence quality using GRADE criteria
- Communicate evidence honestly to patients

Content:

- Overview of 2022 systematic review (177 RCTs, PMID: 35180526)
- Evidence by condition: back pain, neck pain, migraine, hypertension, diabetes, infertility
- Methodological limitations: small samples, heterogeneity, blinding challenges
- GRADE evidence quality ratings applied to cupping research
- How to read a scientific paper: structure, statistics, interpretation
- Communicating uncertainty: “the evidence suggests” vs. “the evidence proves”

Signature Elements:

- Sunnah Remedies Evidence & Sunnah Matrix
- Sunnah Remedies Evidence Update: Latest systematic reviews
- Sunnah Remedies Clinical Pearl: Talking to patients about evidence
- Faith & Practice Reflection

Lesson 2.2 — Proposed Mechanisms of Action (4 hours)

Learning Objectives:

- Explain the major proposed mechanisms for cupping’s therapeutic effects
- Distinguish between established, probable, and theoretical mechanisms
- Integrate mechanism understanding with clinical reasoning

Content:

- Taibah Theory: Causative Pathological Substances (CPS) filtration
- Nitric oxide release and reactive hyperemia
- Immune modulation and reticuloendothelial system stimulation
- Myofascial Decompression (MFD): fascial layer separation
- Lymphatic stimulation and drainage enhancement
- Gate control theory and pain modulation
- Placebo considerations: honest discussion

Signature Elements:

- Sunnah Remedies Clinical Pearl: The Mechanisms Framework
- Sunnah Remedies Evidence Update: Mechanism research 2023-2025
- Sunnah Remedies Comparison Table: Mechanisms by condition
- Faith & Practice Reflection

Lesson 2.3 — Physiological Effects on Body Systems (4 hours)**Learning Objectives:**

- Describe how cupping affects cardiovascular, musculoskeletal, neurological, immune, and metabolic systems
- Apply physiological knowledge to treatment planning
- Monitor for physiological responses during treatment

Content:

- Cardiovascular: blood pressure effects, microcirculation, cholesterol
- Musculoskeletal: myofascial release, flexibility, pain reduction
- Neurological: pain gate modulation, endorphin release, migraine pathways
- Immunological: white blood cell activity, lymphatic flow
- Metabolic: glucose regulation, detoxification concepts
- Homeostasis: supporting the body's self-regulatory capacity
- Skin physiology: effects on dermal and subdermal structures

Signature Elements:

- Sunnah Remedies Infographic: Body Systems & Cupping
- Sunnah Remedies Clinical Pearl: Physiological monitoring during treatment
- Sunnah Remedies Evidence Update: System-specific research
- Faith & Practice Reflection

Lesson 2.4 — Blood Analysis and Dark Field Microscopy (4 hours)

Learning Objectives:

- Understand the principles of dark field microscopy as applied to cupping
- Evaluate claims about blood analysis with critical scientific literacy
- Communicate honestly about what blood analysis can and cannot demonstrate

Content:

- Principles of dark field microscopy: technique and limitations
- Live blood analysis: what it shows, what it doesn't show
- Dried blood analysis: patterns and interpretation
- Research by Dr. David Parker: findings and limitations
- Rouleaux formation, fibrin, toxic metals: scientific context
- The gap between observation and clinical significance
- Ethical communication: avoiding unsubstantiated claims

Signature Elements:

- Sunnah Remedies Evidence Update: Blood analysis research
 - Sunnah Remedies Clinical Pearl: Communicating limitations honestly
 - Sunnah Remedies Common Mistakes: Overinterpreting blood findings
 - Faith & Practice Reflection
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MODULE 3: Human Anatomy for Practitioners

20 guided learning hours | 5 lessons

Lesson 3.1 — Skin Anatomy and Histology (4 hours)

Learning Objectives:

- Identify the layers of the skin relevant to cupping practice
- Determine appropriate scarification depth for different body sites
- Recognise skin conditions that affect treatment decisions

Content:

- Epidermis: structure, thickness variation, barrier function
- Dermis: papillary and reticular layers, capillary networks
- Hypodermis: subcutaneous tissue, adipose layer
- Skin thickness by body site: clinical implications for incision depth
- Skin conditions: acne, eczema, psoriasis, keloid scarring, thin/fragile skin
- Age-related skin changes: implications for elderly patients

- Wound healing phases: how cupping incisions heal

Signature Elements:

- Sunnah Remedies Medical Illustration: Skin Cross-Section
- Sunnah Remedies Clinical Pearl: Scarification Depth Guide
- Sunnah Remedies Common Mistakes: Incorrect depth complications
- Faith & Practice Reflection

Lesson 3.2 — The Circulatory System (4 hours)

Learning Objectives:

- Identify major blood vessels and their relevance to cupping
- Understand capillary structure and function
- Apply vascular knowledge to safe cup placement

Content:

- Arteries, arterioles, capillaries, venules, veins: structure and function
- Capillary networks in the skin: the target of wet cupping
- Endothelial function and vascular tone
- Blood composition: cells, plasma, proteins
- Major vessels to avoid: arteries, deep veins, varicose veins
- Circulatory changes during cupping: what happens and why

Signature Elements:

- Sunnah Remedies Medical Illustration: Superficial Vascular Anatomy
- Sunnah Remedies Clinical Pearl: Vessel Avoidance Map
- Sunnah Remedies Practitioner Tip: Palpation before placement
- Faith & Practice Reflection

Lesson 3.3 — The Heart and Cardiovascular Function (4 hours)

Learning Objectives:

- Understand cardiac structure and function
- Recognise cardiovascular contraindications and precautions
- Monitor cardiac safety during treatment

Content:

- Heart structure: chambers, valves, coronary circulation
- Cardiac cycle and blood pressure regulation
- Common cardiovascular conditions: hypertension, hypotension, arrhythmias
- Heart failure: implications for cupping

- Blood thinners and anticoagulants: mechanism and clinical considerations
- Cardiovascular red flags: when to refer urgently

Signature Elements:

- Sunnah Remedies Medical Illustration: Heart Anatomy
- Sunnah Remedies Clinical Pearl: BP Monitoring Protocol
- Sunnah Remedies Common Mistakes: Ignoring cardiovascular history
- Faith & Practice Reflection

Lesson 3.4 — The Lymphatic System (4 hours)

Learning Objectives:

- Describe lymphatic structure and function
- Explain how cupping affects lymphatic drainage
- Identify lymphatic structures to avoid during treatment

Content:

- Lymphatic vessels, nodes, and organs
- Lymph formation and circulation
- Major lymph node groups: cervical, axillary, inguinal
- The thoracic duct and right lymphatic duct
- Lymphatic oedema: causes and considerations
- Cupping and lymphatic stimulation: evidence and mechanism
- Areas to avoid: lymph node regions

Signature Elements:

- Sunnah Remedies Medical Illustration: Lymphatic System
- Sunnah Remedies Clinical Pearl: Lymphatic Safety Zones
- Sunnah Remedies Evidence Update: Lymphatic research
- Faith & Practice Reflection

Lesson 3.5 — Fascia, Muscles, and the Musculoskeletal System (4 hours)

Learning Objectives:

- Understand fascial structure and its relevance to cupping
- Identify major muscles relevant to common cupping sites
- Apply anatomical knowledge to precise cup placement

Content:

- Fascia: superficial, deep, visceral — structure and function
- Myofascial pain syndrome and trigger points

- Major muscles for cupping: trapezius, erector spinae, latissimus dorsi, rhomboids, gluteals
- Muscle attachments, fibre directions, and trigger point locations
- Myofascial Decompression (MFD): the science of negative pressure on fascia
- Common musculoskeletal conditions treated with cupping

Signature Elements:

- Sunnah Remedies Medical Illustration: Fascial Layers and Muscle Groups
 - Sunnah Remedies Clinical Pearl: Muscle-Based Cup Placement Guide
 - Sunnah Remedies Common Mistakes: Poor anatomical placement
 - Faith & Practice Reflection
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MODULE 4: Clinical Practice & Safety

20 guided learning hours | 5 lessons

Lesson 4.1 — Equipment, Instruments, and Supplies (4 hours)

Learning Objectives:

- Select appropriate equipment for wet cupping
- Understand regulatory requirements for medical devices
- Maintain equipment to clinical standards

Content:

- Cup types: plastic disposable, glass, silicone, bamboo — advantages and limitations
- Pump systems: manual, electric, pressure gauges
- Sterile lancets: types, gauges, single-use requirements
- Gloves, antiseptics, gauze, dressings: selection and use
- Sharps containers and clinical waste disposal
- Equipment sterilisation: autoclave, chemical, disposable
- Regulatory compliance: CE marking, FDA clearance, local requirements

Signature Elements:

- Sunnah Remedies Equipment Checklist
- Sunnah Remedies Clinical Pearl: The Sunnah Remedies SAFE Hijama Protocol (Equipment)
- Sunnah Remedies Common Mistakes: Reusing single-use items
- Faith & Practice Reflection

Lesson 4.2 — Infection Prevention and Control (4 hours)

Learning Objectives:

- Apply universal precautions and standard infection control measures
- Implement a sterilisation protocol appropriate to wet cupping
- Manage clinical waste according to regulatory requirements

Content:

- Chain of infection: source, transmission, susceptible host
- Universal precautions: hand hygiene, PPE, safe sharps handling
- Sterilisation hierarchy: critical, semi-critical, non-critical items
- Environmental cleaning: surfaces, equipment, treatment room
- Waste management: sharps, clinical waste, general waste
- Blood-borne pathogens: HIV, Hepatitis B and C, risks and precautions
- Clinic layout: clean and dirty zones, workflow design

Signature Elements:

- Sunnah Remedies Infection Control Standard
- Sunnah Remedies Clinical Pearl: The Clean-to-Dirty Workflow
- Sunnah Remedies Common Mistakes: Cross-contamination errors
- Faith & Practice Reflection

Lesson 4.3 — Patient Assessment and Clinical Reasoning (4 hours)

Learning Objectives:

- Conduct a comprehensive pre-treatment assessment
- Identify red flags requiring medical referral
- Apply clinical reasoning to treatment planning

Content:

- The Sunnah Remedies 7-Step Patient Assessment:
 1. Presenting complaint and goals
 2. Medical history (systematic review)
 3. Current medications and allergies
 4. Previous Hijama experience
 5. Lifestyle factors
 6. Vital signs
 7. Risk stratification and contraindication screening

- Red flags: unexplained weight loss, persistent fever, new neurological symptoms, suspected malignancy
- When to refer: clear referral pathways
- Clinical reasoning: from assessment to treatment plan
- Documentation standards

Signature Elements:

- Sunnah Remedies Clinical Pearl: The 7-Step Patient Assessment
- Sunnah Remedies Patient Communication Box: Explaining assessment to patients
- Sunnah Remedies Common Mistakes: Inadequate screening
- Faith & Practice Reflection

Lesson 4.4 — Contraindications and Risk Stratification (4 hours)

Learning Objectives:

- Identify absolute and relative contraindications to wet cupping
- Perform individual risk-benefit assessment
- Make appropriate clinical decisions in borderline cases

Content:

- Absolute contraindications: haemophilia, severe anaemia, active infection, suspected DVT, pregnancy, patient refusal
- Relative contraindications: diabetes, hypertension, epilepsy, immunocompromise, children, elderly, anticoagulants
- Condition-specific considerations: diabetes (glucose monitoring), hypertension (BP thresholds), epilepsy (seizure risk)
- The risk stratification framework: a structured decision tool
- Documenting clinical reasoning
- When to consult the patient's GP
- Trial cupping: protocol for borderline cases

Signature Elements:

- Sunnah Remedies Contraindication Decision Tree
- Sunnah Remedies Clinical Pearl: The Risk Stratification Framework
- Sunnah Remedies Common Mistakes: Treating without proper screening
- Faith & Practice Reflection

Lesson 4.5 — Consent, Documentation, and Legal Framework (4 hours)

Learning Objectives:

- Obtain valid informed consent

- Maintain clinical documentation to legal standards
- Understand the legal framework governing complementary therapy practice

Content:

- Informed consent: elements, process, documentation
- Capacity to consent: adults, children, vulnerable adults
- GDPR and patient data protection
- Clinical records: what to document, how long to keep
- Photography consent: before/after images
- Professional indemnity insurance: requirements and coverage
- Legal liability: negligence, scope of practice, advertising standards
- Complaints handling and incident reporting

Signature Elements:

- Sunnah Remedies Clinical Pearl: The Consent Conversation Framework
- Sunnah Remedies Patient Communication Box: Explaining risks clearly
- Sunnah Remedies Professional Standards: Documentation Checklist
- Faith & Practice Reflection

MODULE 5: Treatment Protocols

24 guided learning hours | 6 lessons

Lesson 5.1 — Sunnah Points: Anatomical Landmarks (4 hours)

Learning Objectives:

- Locate the five primary Sunnah points with anatomical precision
- Understand the Prophetic basis for each point
- Apply Sunnah points within a broader treatment plan

Content:

- Al-Kahil (DU14/DU15 region): between the shoulder blades — the primary point
- Akhda'een: the two veins at the sides of the neck
- Al-Hamma: the vertex (top of the head)
- Umm al-Muqeeth: the occipital region
- Al-Katifain (GB21): the shoulder points
- Authentic Hadith evidence for each point
- Scholarly commentary on Sunnah point selection
- Integrating Sunnah points with clinical points

Signature Elements:

- Sunnah Remedies Medical Illustration: Sunnah Points Anatomical Map
- Sunnah Remedies Islamic Insight: The Prophetic Basis for Each Point
- Sunnah Remedies Clinical Pearl: Sunnah Points in Modern Practice
- Faith & Practice Reflection

Lesson 5.2 — Standard Health Detox Protocol (4 hours)**Learning Objectives:**

- Apply the standard detox protocol safely and effectively
- Individualise treatment parameters based on patient assessment
- Monitor treatment response and adjust accordingly

Content:

- The golden rule: place cups on or near the source of concern
- Standard parameters: 5-7 cups, -0.05 to -0.08 MPa, 50-150ml total
- Frequency: monthly maintenance, weekly for acute conditions
- Individualisation: age, condition, skin type, tolerance
- The Sunnah Remedies SAFE Hijama Protocol: Systematic, Anatomical, Faithful, Evidence-informed
- Treatment planning: from assessment to protocol selection
- Monitoring and follow-up

Signature Elements:

- Sunnah Remedies SAFE Hijama Protocol (full framework)
- Sunnah Remedies Clinical Pearl: Treatment Parameter Quick Reference
- Sunnah Remedies Common Mistakes: One-size-fits-all approach
- Faith & Practice Reflection

Lesson 5.3 — Wet Cupping Procedure: Step-by-Step (4 hours)**Learning Objectives:**

- Perform the complete wet cupping procedure with technical precision
- Execute each step with correct technique, timing, and safety measures
- Adapt technique for different body sites and patient presentations

Content:

- Step 1: Preparation (consent verification, positioning, draping, skin prep)
- Step 2: Cup placement (anatomical landmarks, suction application)
- Step 3: Waiting period (3-5 minutes, skin monitoring)

- Step 4: Cup removal (controlled release)
- Step 5: Scarification (0.5-1mm depth, 5-7mm length, 3-5 incisions)
- Step 6: Reapplication (moderate suction over incisions)
- Step 7: Blood collection (monitoring colour change)
- Step 8: Completion (removal, cleaning, dressing)
- Step 9: Aftercare (instructions, follow-up scheduling)
- Technique variations by body site
- Troubleshooting: poor flow, excessive bleeding, patient discomfort

Signature Elements:

- Sunnah Remedies Clinical Procedure Visual Guide
- Sunnah Remedies Clinical Pearl: Scarification Technique Mastery
- Sunnah Remedies Common Mistakes: Technical errors and their consequences
- Faith & Practice Reflection

Lesson 5.4 — Condition-Specific Applications (4 hours)

Learning Objectives:

- Apply Hijama for common conditions with evidence-informed practice
- Communicate honestly about evidence strengths and limitations
- Integrate Hijama with conventional care appropriately

Content:

- Musculoskeletal: back pain, neck pain, myofascial pain, sports injuries
- Metabolic: Type 2 diabetes (adjunct), dyslipidaemia
- Cardiovascular: hypertension (short-term evidence)
- Reproductive: female infertility (preliminary evidence)
- Neurological: migraine, tension headaches
- Psychological: anxiety, depression (limited evidence)
- General wellness and detoxification
- Evidence grading for each application
- Honest communication: what we know, what we don't know

Signature Elements:

- Sunnah Remedies Evidence & Sunnah Matrix: Conditions
- Sunnah Remedies Clinical Pearl: Treatment Planning by Condition
- Sunnah Remedies Patient Communication Box: Discussing evidence with patients
- Faith & Practice Reflection

Lesson 5.5 — Special Populations (4 hours)

Learning Objectives:

- Adapt Hijama practice for special populations
- Manage specific risks associated with diabetes, hypertension, epilepsy, pregnancy, childhood, and elderly patients
- Make appropriate clinical decisions for complex cases

Content:

- Diabetic patients: glucose monitoring, delayed healing, infection risk
- Hypertensive patients: BP thresholds, monitoring protocol
- Epilepsy: seizure risk, environment safety
- Elderly patients: skin fragility, reduced suction, medication review
- Children (16+): consent, guardian involvement, reduced parameters
- Women's health: menstruation, pregnancy (absolute contraindication), postpartum
- Anticoagulant patients: risk assessment, trial protocol
- Immunocompromised patients: enhanced precautions

Signature Elements:

- Sunnah Remedies Special Populations Decision Tree
- Sunnah Remedies Clinical Pearl: Diabetic Patient Protocol
- Sunnah Remedies Common Mistakes: Treating without population-specific precautions
- Faith & Practice Reflection

Lesson 5.6 — Managing Complications and Aftercare (4 hours)

Learning Objectives:

- Recognise and manage common complications
- Provide comprehensive aftercare instructions
- Know when and how to seek emergency assistance

Content:

- Expected responses: marks, itching, fatigue, tenderness
- Excessive bleeding: recognition and management
- Syncope (vasovagal response): prevention and management
- Infection: recognition, documentation, referral
- Burns (fire cupping): immediate management
- Scarring: prevention and risk factors
- Aftercare instructions: written and verbal

- Emergency procedures: when and how to call for help
- Follow-up scheduling and monitoring

Signature Elements:

- Sunnah Remedies Aftercare Protocol
 - Sunnah Remedies Clinical Pearl: Syncope Prevention and Management
 - Sunnah Remedies Common Mistakes: Inadequate aftercare
 - Faith & Practice Reflection
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MODULE 6: Professional Standards & Practice

16 guided learning hours | 4 lessons

Lesson 6.1 — Understanding Hijama Marks vs. Bruising (4 hours)

Learning Objectives:

- Distinguish between Hijama marks and traumatic bruising
- Interpret mark colour and characteristics
- Educate patients effectively about what to expect

Content:

- The anatomy of a Hijama mark: capillary dilation, not rupture
- Colour interpretation: pink, red, purple, black — what each indicates
- Timeline: from application to complete resolution (4-10 days)
- Bruising vs. marks: clinical differences table
- Patient education: what to tell patients before treatment
- Photographic documentation: consent and technique
- When marks indicate a problem

Signature Elements:

- Sunnah Remedies Infographic: Mark Progression Timeline
- Sunnah Remedies Patient Communication Box: Explaining marks to patients
- Sunnah Remedies Clinical Pearl: Mark Photography for Records
- Faith & Practice Reflection

Lesson 6.2 — Fasting and Hijama: Scholarly Guidance (4 hours)

Learning Objectives:

- Explain the scholarly positions on Hijama during fasting
- Apply evidence-based clinical guidance for fasting patients

- Navigate Ramadan practice with confidence

Content:

- Authentic Hadith: Bukhari 5694, Bukhari 1940
- Scholarly positions by madhhab: Maliki, Shafi'i, Hanafi, Hanbali
- The majority view: Hijama does not break the fast
- Clinical considerations: hypoglycaemia, dehydration, weakness
- Practical Ramadan guidance: scheduling, hydration, lighter protocols
- Documenting fasting status in clinical records
- Respecting patient choice and scholarly preference

Signature Elements:

- Sunnah Remedies Islamic Insight: Scholarly Positions Summary
- Sunnah Remedies Clinical Pearl: Ramadan Treatment Protocol
- Sunnah Remedies Patient Communication Box: Discussing fasting
- Faith & Practice Reflection

Lesson 6.3 — Sunnah Remedies: Complementary Prophetic Medicine (4 hours)

Learning Objectives:

- Identify key remedies from the Prophetic tradition
- Understand the evidence (traditional and scientific) for each remedy
- Recommend complementary remedies within scope of practice

Content:

- Honey: antimicrobial, wound healing (Bukhari 5680)
- Black Seed (Nigella sativa): immunomodulatory, anti-inflammatory
- Talbina (barley): gastrointestinal soothing (Bukhari 5325)
- Qist al-Hindi (Costus): respiratory conditions (Bukhari 5713)
- Senna: digestive cleansing (Ibn Majah 3457)
- Zamzam water: spiritual and physical nourishment
- Olive oil: cardiovascular and skin health
- Dates (Ajwa): nutritional density, energy
- Scope of practice: what practitioners can recommend

Signature Elements:

- Sunnah Remedies Infographic: Prophetic Remedies at a Glance
- Sunnah Remedies Islamic Insight: Hadith for Each Remedy
- Sunnah Remedies Evidence Update: Modern research on prophetic remedies

- Faith & Practice Reflection

Lesson 6.4 — Building Your Practice (4 hours)

Learning Objectives:

- Set up a Hijama clinic to professional standards
- Understand business fundamentals for independent practice
- Market services ethically and effectively

Content:

- Clinic setup: room requirements, equipment, ventilation, lighting
- Professional standards: CMA/IPHM accreditation, CPD requirements
- Insurance: professional indemnity, public liability, employer's liability
- Pricing structure: consultation fees, treatment packages
- Ethical marketing: what you can and cannot claim
- Patient retention: communication, follow-up, packages
- Record-keeping systems: paper and digital
- Continuing professional development: pathways and requirements
- Building referral networks with GPs and other healthcare professionals

Signature Elements:

- Sunnah Remedies Clinic Setup Checklist
- Sunnah Remedies Professional Standards: CPD Framework
- Sunnah Remedies Patient Communication Box: Ethical marketing language
- Faith & Practice Reflection

SECTION D: ASSESSMENT STRATEGY

D.1 Assessment Philosophy

Assessment at Sunnah Remedies Academy serves three purposes:

1. **To protect patients** — by ensuring only competent practitioners are certified
2. **To validate learning** — by confirming students have achieved the learning outcomes
3. **To guide development** — by identifying areas for continued growth

Assessment is rigorous, fair, and aligned with the learning outcomes. Students must demonstrate competence in ALL domains to achieve certification — there is no compensation between domains.

D.2 Assessment Components

Component 1: Written Examination (40% of final grade)

Format: 120-minute closed-book examination **Pass mark:** 70% **Content:**

- 60 multiple-choice questions (knowledge and application)
- 10 short-answer questions (clinical reasoning)
- 2 case study analyses (integration and decision-making)

Domains assessed: K1-K7, P1-P2, I1-I4

Component 2: Practical OSCE (40% of final grade)

Format: 5 stations, 20 minutes each **Pass mark:** 70% per station (must pass all stations) **Stations:**

1. Patient assessment and clinical reasoning
2. Informed consent and communication
3. Wet cupping procedure (full technique)
4. Adverse event management (simulated emergency)
5. Documentation and aftercare

Domains assessed: S1-S7, P3, P4

Component 3: Clinical Portfolio (20% of final grade)

Format: Submission of 20 supervised case studies **Pass mark:** 70% **Requirements:**

- Complete documentation for each case
- Variety of conditions and patient presentations
- Evidence of clinical reasoning
- Reflective commentary on learning from each case
- Supervisor sign-off for each case

Domains assessed: S1-S7, P1-P6, I1-I4

D.3 Certification Pathway

Stage	Requirement	Timeline
Enrolment	Application review, prerequisites verified	Before course start
Module Completion	Attend all 24 lessons, complete all knowledge checks	Weeks 1-12
Practical Workshops	Attend all supervised practice sessions	Weeks 4-12
Written Examination	Achieve 70% or higher	Week 13

Table 4 – continued

Stage	Requirement	Timeline
Practical OSCE	Pass all 5 stations at 70% or higher	Week 13-14
Portfolio Submission	Submit 20 case studies at 70% or higher	Week 14-16
Certification	Award of CHP credential, certificate, and registration	Week 16

Re-sit policy: Students who do not achieve the pass mark may re-sit once at no additional cost within 6 months. A second re-sit incurs a nominal administrative fee.

D.4 CPD Pathway

Certified practitioners are required to maintain their credential through continuing professional development:

CPD Category	Hours Required (per 2 years)	Examples
Clinical Practice	20 hours	Supervised practice, case review, peer observation
Education	10 hours	Workshops, webinars, conference attendance
Research Literacy	5 hours	Journal review, evidence updates, research participation
Islamic Studies	5 hours	Relevant courses, scholarly lectures, personal study

Total: 40 hours per 2-year cycle

Practitioners who do not meet CPD requirements will have their certification suspended until requirements are met.

SECTION E: TEACHING METHODOLOGY

E.1 Adult Learning Principles

Every lesson is designed using evidence-based adult learning principles:

Andragogy (Knowles' Principles)

- Adults need to know WHY they are learning something
- Adults are self-directed and responsible for their own learning

- Adults bring life experience that enriches learning
- Adults are ready to learn what they need to know
- Adults are problem-oriented, not subject-oriented
- Adults are motivated by internal factors

Application:

- Every lesson opens with “Why this matters”
- Learning objectives are explicit and measurable
- Case studies draw on realistic clinical scenarios
- Self-assessment and reflection are built into every lesson
- Practical application is emphasised over theoretical memorisation

E.2 Teaching Methods

Method	Proportion	Description
Live Lecture	30%	Expert-led presentation with visual aids, Q&A, and discussion
Case-Based Learning	20%	Realistic patient scenarios worked through in groups
Practical Demonstration	20%	Instructor demonstration followed by student practice
Supervised Practice	20%	Hands-on practice with individual feedback
Self-Directed Study	10%	Reading, online modules, reflection exercises

E.3 The Sunnah Remedies Lesson Structure

Every lesson follows a consistent structure:

1. **Opening (5 min)** — Why this matters; connection to previous learning
2. **Learning Objectives (2 min)** — What students will know and be able to do
3. **Prior Knowledge Check (5 min)** — Brief assessment of existing understanding
4. **Core Teaching (40-60 min)** — Main content delivery with visual aids
5. **Clinical Scenario (15 min)** — Case study application
6. **Knowledge Check (10 min)** — Formative assessment
7. **Practical Application (15-20 min)** — Hands-on or simulated activity
8. **Reflection (5 min)** — Self-assessment and Islamic integration
9. **Summary (3 min)** — Key takeaways and preview of next lesson

SECTION F: RESOURCES AND MATERIALS

F.1 Student Resources

Core Materials:

- Comprehensive slide deck (all 24 lessons)
- Student workbook (exercises, case studies, reflection prompts)
- Practical handbook (step-by-step procedures with photographs)
- Anatomy reference sheets (laminated)
- Quick reference cards (pocket-sized)

Digital Resources:

- Online learning platform (recorded lectures, quizzes, forums)
- Anatomy app subscription
- Research database access
- Islamic reference library

Equipment:

- Starter kit: cups, pump, lancets, gloves, antiseptic, gauze
 - Practice pad for scarification technique
 - Blood pressure monitor
-

F.2 Instructor Resources

- Complete instructor guide (teaching notes, timing, discussion prompts)
 - Assessment rubrics and marking guides
 - OSCE station instructions and checklists
 - Case study bank (50+ scenarios)
 - Practical demonstration videos
 - Islamic reference verification guide
-

F.3 Clinic Resources

- Consent form templates
- Patient medical history form
- Treatment record template
- Contraindication checklist
- Aftercare instruction sheet
- Emergency procedure poster
- Infection control SOP

- Cleaning SOP
 - Sharps handling SOP
 - Clinic setup guide
-

SECTION G: QUALITY ASSURANCE

G.1 Internal Quality Control

- All content reviewed by subject matter experts (clinical, anatomical, Islamic)
- All Islamic references verified against primary sources
- All scientific claims supported by peer-reviewed citations
- All procedures reviewed against current clinical guidelines
- Annual content review and update cycle

G.2 External Validation

- CMA and IPHM accreditation maintained
- Advisory board of clinicians, scholars, and educators
- Student feedback incorporated into continuous improvement
- Graduate outcomes tracked and reported

G.3 Academic Integrity

- All content is original intellectual property of Sunnah Remedies Academy
 - Islamic references verified against Quran.com and Sunnah.com
 - Scientific references from PubMed-indexed journals
 - No copyrighted material reproduced without permission
 - Proper attribution for all external sources
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SECTION H: SIGNATURE FRAMEWORKS

The following proprietary frameworks are the intellectual property of Sunnah Remedies Academy and appear throughout the curriculum:

H.1 The Sunnah Remedies SAFE Hijama Protocol

Systematic | Anatomical | Faithful | Evidence-informed

A four-pillar framework for every treatment:

- **Systematic:** Follow a structured assessment and treatment process every time
- **Anatomical:** Base all cup placement on sound anatomical knowledge
- **Faithful:** Honour the Sunnah while practising within evidence-informed boundaries

- **Evidence-informed:** Integrate the best available evidence with clinical expertise

H.2 The Sunnah Remedies 7-Step Patient Assessment

1. Presenting complaint and goals
2. Medical history (systematic review)
3. Current medications and allergies
4. Previous Hijama experience
5. Lifestyle factors
6. Vital signs
7. Risk stratification and contraindication screening

H.3 The Sunnah Remedies Clinical Excellence Framework

- Knowledge: Know your subject deeply
- Skill: Practise with precision and care
- Safety: Protect every patient always
- Communication: Explain, listen, and respond
- Ethics: Act with integrity in every decision
- Reflection: Learn continuously from every experience

H.4 The Sunnah Remedies Evidence & Sunnah Matrix

A decision-support tool that maps every clinical claim across two dimensions:

- Evidence strength (strong / moderate / preliminary / traditional)
- Sunnah basis (explicit Hadith / scholarly recommendation / traditional practice / clinical experience)

H.5 The Sunnah Remedies Aftercare Protocol

- Immediate: dressing, observation, hydration
- First 24 hours: activity restrictions, wound care
- Days 2-10: mark monitoring, healing assessment
- Follow-up: scheduling, progress review, next treatment planning

H.6 The Sunnah Remedies Infection Control Standard

- Hand hygiene: before and after every patient contact
- PPE: gloves minimum; apron and eye protection when indicated
- Sterile technique: critical items single-use; semi-critical high-level disinfection
- Environmental cleaning: between patients, daily, weekly
- Waste management: segregated, labelled, licensed disposal
- Documentation: record all infection control measures

APPENDIX: DOCUMENT CONTROL

Version	Date	Author	Changes
1.0	[Date]	Sunnah Remedies Academy	Initial blueprint
2.0	[Date]	Sunnah Remedies Academy	Comprehensive revision following Version 1 review

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